

1. Administrative & Study Identification

Full Study Title: The EVOLVE study: A prospective, observational study to evaluate the patient-centric outcomes over 2 years of treatment with Tezepelumab in the real-life clinical practice in Greece
Short Title/Acronym: EVOLVE
Protocol Number: D5180R00032
NCT Number: NCT06724575
Sponsor Name: AstraZeneca
Investigational Product: Tezepelumab
Phase of Development: N/A (Observational / Real-world evidence)
Medical Monitor: AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To describe the patient-reported asthma symptom control using the Asthma Control Questionnaire (ACQ-6) at baseline and up to 104 weeks post-index.
Secondary Objectives: To assess quality of life, lung function, exacerbation rate, corticosteroid use, health-care utilization, and evaluate novel composite endpoints including early clinically important improvement, disease stability, super-response, and clinical remission at 2 years.

2.2 Background & Rationale Tezepelumab is a first-in-class human monoclonal antibody for uncontrolled severe asthma that acts at the top of the inflammatory cascade by targeting thymic stromal lymphopoietin (TSLP). Considering its recent addition to the severe asthma therapeutic arsenal, there is a need for real-world evidence to inform treatment-decision making in clinical practice and support regulatory decisions. EVOLVE is designed to generate evidence on patient-reported outcomes under pragmatic clinical conditions over a 2-year period in Greece.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Prospective cohort) **Control Type:** Single-arm (Real-world practice) **Blinding:** Open-label **Randomization:** N/A (Allocation planned strategically: 70% Type 2-high / 30% Type 2-low; 80% biologic naïve / 20% switching)

3.2 Planned Enrollment & Duration Target \$N\$: 150 adult patients
Number of Sites: 20-25 healthcare settings in Greece **Estimated**

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male or female outpatients aged 12 years or older at the time of signing the informed consent/assent form. 2. Patients who have been prescribed but not yet initiated treatment with tezepelumab according to the label and local market reimbursement criteria. 3. Diagnosis of asthma for at least 52 weeks prior to enrolment date and symptoms confirmed by the Investigator not to be due to alternative diagnoses. 4. Receipt of at least one prescription of high-dose Inhaled Corticosteroids (ICS) during the 52 weeks prior to enrolment date.

4.2 Exclusion Criteria 1. Any contraindication to tezepelumab as per the approved product label or in the opinion of the Investigator. 2. Concurrent or recent (<30 days before the index date) treatment with biologics for asthma, with the exception of stable allergen immunotherapy. 3. Participation in an observational study that might influence the assessment for the current study, or participation in an interventional clinical trial in the last 3 months. 4. Currently pregnant (or intention to become pregnant) or lactating women.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Tezepelumab 210 mg administered every 4 weeks (prescribed as per standard clinical practice / approved label). **Route of Administration:** Subcutaneous (SC) injection. **Duration of Treatment:** Up to 104 weeks (2 years) of follow-up observation.

5.2 Concomitant & Prohibited Therapy Permitted: Standard asthma maintenance controller medications (including high-dose ICS) and stable allergen immunotherapy. **Prohibited:** Concurrent treatment with other monoclonal antibodies/biologics for asthma.

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :---
| :--- | :--- | | **Screening/Baseline** | Day 0 | Informed Consent, Medical History, Tezepelumab Initiation, Baseline ACQ-6 | |
Interim 1 | Week 4 | Patient-reported outcomes (PROs), Symptom monitoring | | **Interim 2-4** | Weeks 12, 24, 52 | ACQ-6, Lung function evaluation, Exacerbation assessment | | **Interim 5** | Week 72 | Continuation of real-world outcome data collection | | **End of Study** | Week 104 | Final PROs, Clinical remission evaluation, Healthcare utilization review |

(Note: Data will be collected through visits as per standard clinical practice in routine care settings.)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Patient-reported asthma symptom control up to 104 weeks post-index. **Measurement Method:** Evaluated using the Asthma Control Questionnaire (ACQ-6).

7.2 Secondary & Exploratory Endpoints * Assessment of quality of life, lung function, exacerbation rate, corticosteroid use, and health-care utilization. * Novel composite endpoints: early clinically important improvement, disease stability, super-response, and clinical remission at 2 years.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard real-world pharmacovigilance collection via routine clinical practice visits. **Serious Adverse Events (SAEs):** Reported within 24 hours to regulatory authorities and the sponsor, consistent with routine pharmacovigilance for post-marketing biologics. **Data Monitoring Committee (DMC):** Managed internally by the sponsor's observational research and safety team.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Real-World Evidence (RWE) Set / Intent-to-Treat (all patients meeting eligibility who receive at least one dose of tezepelumab). **Sample Size Justification:** \$N=150\$ adult patients is targeted to provide a robust representative sample across 20-25 healthcare settings, appropriately stratified (70% Type 2-high / 30% Type 2-low; 80% biologic naïve / 20% switching) to reflect real-world Greek clinical practice. **Handling of Missing Data:** Standard multiple imputation or available-case analysis appropriate for prospective observational registries.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Pharmacoepidemiology Practices (GPP), Good Clinical Practice (GCP), and the Declaration of Helsinki. **Monitoring Plan:** Data collected prospectively during routine medical visits. **Audit Trail:** Electronic Case Report Form (eCRF) utilizing source data verification against standard medical records.

11. Study Identifiers & Governance

Primary ID: D5180R00032 **Registry Number:** NCT06724575 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** EVOLVE **Official Regulatory Title:** The EVOLVE study: A prospective, observational study to evaluate the patient-centric outcomes over 2 years of treatment with Tezepelumab in the real-life clinical practice in Greece

12. Clinical Framework (Severe Asthma Specific)

Primary Condition: Severe Asthma **Diagnosis Threshold:** Confirmed asthma $\geq$ 52 weeks; symptomatic despite high-dose Inhaled Corticosteroids (ICS) **Medical Methodology:** Standard clinical practice prescribing of human monoclonal antibody (anti-TSLP) **Optimization Target:** Improvement in asthma symptom control and clinical remission at 2 years

13. Study Procedures & Methodology

Management Pathway: Standard routine care setting for severe asthma **Education Protocol (HCP):** Conducted according to the local approved label for Tezepelumab **Education Protocol (Patient):** Self-reporting training for the Asthma Control Questionnaire (ACQ-6) **Quality Audit Mechanism:** Continuous collection of Patient-Reported Outcomes (PROs) and healthcare resource utilization tracking during standard clinical visits

14. Observation & Follow-Up Schedule

Total Duration: 104 Follow-up weeks (2 years) **Onsite Visit Cadence:** Baseline (Enrollment) and Weeks 4, 12, 24, 52, 72, and 104 **Remote Monitoring Frequency:** N/A (Driven by routine clinical practice visits) **Checklist Review Interval:** ACQ-6 completion at all predefined follow-up time points

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				